

Southern California CSU DNP Consortium

California State University, Fullerton
California State University, Long Beach
California State University, Los Angeles

AN EVIDENCE-BASED INTERVENTION TO ADDRESS
BURNOUT AMONG HOSPICE NURSES

A DOCTORAL PROJECT

Submitted in Partial Fulfillment of the Requirements

For the degree of

DOCTOR OF NURSING PRACTICE

By

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May 2023

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DOI 10.5281/zenodo.10408709
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ABSTRACT

Background: Work-related traumatic events can lead to burnout, particularly among nurses caring for high-acuity patients. Hospice nurses are exposed to work-related traumatic events due to taking care of terminally ill patients. Hospice nurses typically do not have adequate time to grieve the loss of their patients or access to organizational resources due to working in a community setting. Burnout is defined as increased feeling of exhaustion and disengagement, where nurses are withdrawn from providing high-quality work. Burnout and the associated exhaustion and disengagement negatively impact nurses, patients, and healthcare organizations. **Purpose:** The purpose of this Doctor of Nursing Practice (DNP) project was to examine if a debriefing protocol activated following a job-related traumatic event would decrease burnout defined by level of exhaustion and disengagement in nurses working in a Hospice Agency in Southern California. The Debriefing Protocol was designed for this project and Hospice agency, which has an average daily census of 290 patients. **Methods/Measure:** The Oldenburg Burnout Inventory (OLBI) was used to measure overall burnout, exhaustion, and disengagement, which was administered online via RedCAP before and after implementing the Debriefing Protocol. Before implementation, hospice nurses were educated on the purpose of the Debriefing Protocol. **Data Analysis:** Data were analyzed using SPSS Statistical Software (version 28.0.0.0). We compared overall burnout as measured by levels of exhaustion and disengagement before and after implementing the debrief protocol. All comparisons were performed using dependent sample *t*-tests with Bonferroni corrections with a significance level set at $P < 0.05$. There was no difference in burnout, exhaustion, or disengagement as a result of the Debriefing Protocol. **Discussion:** Potential reasons could be the fidelity to intervention problem (Not enough time between education and training of intervention facilitators to the time of implementation, which might have affected how the intervention was delivered. General debriefing was occurring weekly for the pediatric hospice team at the same time; this might have created confusion among activating the debriefing protocol. **Conclusion:** It is imperative to continue to test strategies to combat burnout among nurses. Successful strategies can support hospice nurses and a safe space to become vulnerable and learn from their experiences.

Keywords: hospice nurse, palliative, burnout, code lavender, compassion fatigue, patient satisfaction, employee satisfaction, crisis intervention, meditation, debriefing tools, traumatic stress, resilience, and nurse turnover.

TABLE OF CONTENTS

ABSTRACT.....	iii
ACKNOWLEDGEMENTS.....	vii
BACKGROUND	1
Problem Statement.....	2
Purpose Statement.....	2
Supporting Framework	3
Identifying the Triggering Issues or Opportunities.....	3
Purpose Statement.....	4
Decision Point: Determining if the Issue is a Priority	4
Team Formation.....	4
Assemble, Appraise, Synthesize the Body of Evidence and Second Decision Point	5
Design and Pilot of the Practice Change.....	5
Integrating, Sustaining, Educating, and the Third Decision Point.....	5
Dissemination of Results	6
REVIEW OF LITERATURE	7
Overview.....	7
Secondary Traumatic Stress and Burnout.....	8
Traumatic Events: Impact on Nurses	10
Traumatic Events: Impact on Patients	11
Traumatic Events: Impact on Organizations.....	11
Strategies to Assist After Traumatic Events	11
DESCRIPTION OF INNOVATION	13
The Debriefing Protocol	13
The Algorithm and Decision Points.....	13
The Hospice and Palliative Care Debriefing Guidance Tool.....	14
The Debriefing Tool	15
METHODS	17
Design	17
Setting	17

Sample.....	18
Staff Education.....	19
Procedures.....	19
Approval Process and Ethical Considerations	19
Survey Timeframe	20
Measures	21
Timeline	22
RESULTS	23
Data Analysis	23
DISCUSSION	24
Evaluation	24
Limitations	26
Recommendations.....	27
Conclusions.....	27
REFERENCES	29
APPENDIX A: The Iowa Model	36
APPENDIX B: Articles Searched from CINAHL/EBSCO.....	37
APPENDIX C: Articles Searched from Science Direct	38
APPENDIX D: Articles Searched from PubMed	39
APPENDIX E: The Debrief Protocol: <i>Algorithm for Managers and AOCs</i>	40
APPENDIX F: Hospice and Palliative Care Team Debrief Guidance Tool.....	41
APPENDIX G: Hospice and Palliative Care Team Debrief Process.....	42
APPENDIX H: The Oldenburg Inventory Tool	43
APPENDIX I: Permission to use the Iowa Model.....	44
APPENDIX J: Permission to use the 7 Phases Debriefing Tool	45
APPENDIX K: Institutional Approval Letter to use the Project Site	46
APPENDIX L: DNP Project Timeline	47
APPENDIX M: Demographic Characteristics of the Sample	48

APPENDIX N: Results: Average Burnout Before and After the Intervention.....	49
APPENDIX O: Results: Average Exhaustion Before and After the Intervention.....	50
APPENDIX P: Results: Average Disengagement Before and After the Intervention.....	51

ACKNOWLEDGEMENTS

I want to thank Cohort 10 for being one of the most humble and supportive colleagues I have gone to school with. We have created a bond, made memories, and celebrated milestones in the past five semesters. Dr. Armendariz, thank you for continuously supporting and assisting me with writing this paper, and Dr. Pearce for being my team member, APA, and reference expert. Most importantly, to Dr. Al-Majid. I can't express my appreciation and gratitude this whole year. Your continued belief, encouragement, and support while helping me achieve my goals greatly aided my success in this program. Thank you for your guidance in making me a better version of myself.

Thank you to my leaders at work for allowing me to dedicate time to complete my degree. I can't wait to share what I learned with our organization. Thank you to my family and friends for understanding why I sometimes miss events. To my dad, we did it! Thank you for always being there for me! Mom and Mama would have been very proud if they were with us today. To my husband, Charles, and my children, Cameron, Camila, and Coltrane, thank you for being patient and believing I could do this when I wanted to give up. I love you. Completing this project would not have been possible without all of you. I dedicate this work to you.

Background

Nurses working with end-of-life patients are exposed to work-related traumatic events (Curry, 2021). Repeated exposure to such events results in traumatic stress, anxiety, and depression from witnessing patients' death, grief, pain, and suffering. Nurses' inability to process the experience of a traumatic event and the unacknowledged emotional and psychological distress can lead to increased burnout rates and decreased quality of care provided to patients. Burnout is experienced by 95% of nurses today (American Psychological Association [APA], 2022; Curry, 2021). Additionally, in the United States, 250,000 patient deaths can be attributed to decreased quality of care (Garcia et al., 2019). Hospice staff is more likely to experience burnout and compassion fatigue, which can lead to exhaustion due to their high-acuity work setting and the complexity of patient diagnosis (Cross, 2019; Keidel, 2002). For this paper, burnout, compassion fatigue, and exhaustion can be used interchangeably and further discussed in the literature review. It can be argued that hospice nurses who experience burnout and compassion fatigue may be at an increased risk for providing substandard care and experiencing decreased job satisfaction.

Healthcare organizations and nurse leaders must support strategies to address the effects of traumatic stress and attenuate nurse burnout. The current nursing national turnover rate as of 2022 is estimated at 8.85% to 37% (American Nurse Association [ANA], 2021; Haddad et al., 2022). Research studies showed that 62% of nurses experienced burnout, 20% considered leaving the field, and 30% considered reducing their working hours (Parker, 2019; Vossel, 2021). Approximately 48% of nurses who reported experiencing burnout were actively looking for a different job that was less stressful (Curry, 2021). Ultimately, the high rates of nurses leaving the industry or changing careers will increase the financial strain on healthcare

organizations and systems. A large amount of money is lost in revenue and cost per nurse onboarding, which can amount to nearly \$82,000 without accounting for education and precepting (Schaffer, 2020). When apparent organizational support is present for Hospice Agency staff, the results can reveal increased employee satisfaction, employee engagement, and patient satisfaction. The Hospice Agency can also decrease burnout and secondary traumatic stress (Hotchkiss, 2018; Knott et al., 2023).

Problem Statement

Hospice and palliative care nurses who care for dying or ill patients experience traumatic events caused by their daily witnessing of patient suffering. In 2022, 57% of the Hospice Agency's nurses reported experiencing burnout, and only 86% intend to stay in palliative care for the next 12 months (Willis Towers Watson Survey, 2022). If leaders do not address the traumatic event, the stress and burnout can consequently impact the patient, nurse, and organizational outcomes. Burnout can lead to a feeling of exhaustion, which contributes to disengagement and, subsequently, decreased quality of care. Burnout is defined as an increased feeling of exhaustion and disengagement where nurses are withdrawn from providing high-quality work. The problem this Doctor of Nursing Practice (DNP) evidence-based practice (EBP) project addressed was the stress and burnout experienced by nurses after they encounter a traumatic event at a Southern California Hospice Agency. In this Hospice Agency, there are limited interventions to address traumatic events experienced by staff, which may contribute to the current high incidence of staff burnout and attrition.

Purpose Statement

The purpose of DNP project was to examine if a debriefing protocol activated following a job-related traumatic event would decrease burnout, exhaustion, and disengagement among

nurses working in a Hospice Agency in Southern California. The aim was to measure overall burnout using the Oldenburg Inventory (OLBI), exhaustion, and disengagement before and after the debriefing intervention.

Supporting Framework

Frameworks are used to guide the development of EBP projects (Grove & Gray, 2019). In 1994, a group of nurses and faculty from Iowa Hospital and the University of Iowa created the Iowa Model to guide EBP activities to improve patient care quality. The Iowa Model aimed to provide a method of evaluation to address identified clinical problems in healthcare settings. The Iowa Model was revised to shift the focus from research and application to EBP to guide clinical decisions (Titler et al., 2001). The Iowa Model consists of seven steps and three decision points to conduct evidence-based activities, from identifying a triggering issue to designing, implementing, evaluating, and disseminating results (Appendix A).

Identifying the Triggering Issues or Opportunities

The first step in the Iowa Model was to identify the triggering issues. The Hospice Agency's triggering event was the increased nurse turnover rate and increased flight risk of staff because of the COVID-19 pandemic and burnt-out nurses. As a result, the nurses left working at the Hospice Agency and had to manage a higher census and workloads. Nurse shortages can increase stress levels and Burnout (Essary et al., 2020). Additionally, nurses in the Hospice Agency encountered increased patient deaths due to COVID-19, patient complexity, and family expectations. Dying patients require nurses to provide an elevated level of support for families going through the grieving process. The deaths and the needed assistance with grieving families have exposed nurses to ongoing traumatic events that should be addressed to decrease the effects of stress and burnout.

Purpose Statement

The second step in the Iowa Model was stating the purpose of the project. The purpose of this DNP project was to examine if a debriefing protocol activated following a job-related traumatic event would decrease burnout, exhaustion, and disengagement among nurses working in a Hospice Agency in Southern California. The specific aim was: Using the debriefing protocol would decrease a) overall burnout as measured by the OLBI, b) decrease exhaustion, and c) decrease disengagement.

Decision Point: Determining if the Issue is a Priority

The next step in the Iowa Model was to decide if the proposed change was necessary to address burnout and compassion fatigue. The leadership team and Project Lead identified that addressing burnout and compassion fatigue was a priority for the Hospice Agency. The Debriefing Protocol addressed the traumatic events that lead to stress and burnout among nurses in this Hospice Agency. The Hospice Agency s takes the Willis Towers Watson Caregiver Experience Survey annually. In 2022, the survey results provided the senior leadership team with information on opportunities to improve scores in burnout and intent to stay. The senior leadership convened and determined that addressing traumatic events using the Debriefing Protocol was a priority. They arrived at this decision due to traumatic events' impacting the Hospice Agency, patients, and staff.

Team Formation

The third step in the Iowa Model was to form a team to address the problem. The team recruited consisted of multidisciplinary staff assisting in creating and promoting the implementation of the change and evaluating the results of the DNP project. The team for this project was called the Ethics Taskforce. The team included the Hospice Director (Project Lead),

a clinical manager, a nurse, a Licensed Clinical Social Worker (LCSW), a member of the spiritual care team, the chief mission officer, and the medical director.

Assemble, Appraise, Synthesize the Body of Evidence and Second Decision Point

The next step in the Iowa Model was to search for, retrieve, synthesize, and appraise evidence related to the topic of interest. The Project Lead conducted a comprehensive literature search with the assistance of a librarian at the Pollack Library located at California State University, Fullerton (CSUF). The Project Lead appraised the research results based on their relevance to the DNP project. Based on findings from the literature, the Project Lead determined that there was compelling evidence to support the need to implement the Debriefing Protocol to attenuate burnout and help nurses cope with work-related traumatic events. Moreover, most of the research was related to the acute care setting, identifying a gap in the literature for hospice care settings.

Design and Pilot of the Practice Change

This DNP project involved creating an algorithm to help Clinical Managers or Administrator on Call (AOC) and the affected nurse determine when or if there was a need to activate the Debriefing Protocol after experiencing a traumatic event. The algorithm's development, the Debriefing tool's activation, and step five in the Iowa Model are explained in detail in the methods section of this DNP proposal.

Integrating, Sustaining, Educating, and the Third Decision Point Nurses

Step six of the Iowa Model was integrating and sustaining the Debriefing Protocol. However, the Manager or AOC and Nurse experiencing the traumatic event must answer the third decision point to move on to the sixth step. The sixth step involves determining if the Debriefing Protocol should be adopted and piloted throughout the Hospice Agency. The details

of the approval process, pilot, implementation, and Debriefing Protocol evaluation are explained in this DNP paper's Results and Discussion section.

Dissemination of Results

The last step of the Iowa Model is disseminating the DNP project results. The Project Lead shared the results with the Hospice Agency's sponsoring leaders. The Project Lead will also disseminate the results at the third annual Health and Human Development research showcase and to the faculty and colleagues at CSUF during the formal DNP projects Dissemination Day.

Review of Literature

Overview

This project aimed to implement a Debriefing Protocol to support nurses experiencing work-related traumatic events. A comprehensive search was conducted to identify relevant literature. The following databases were searched: PubMed, CINAHL, EBSCO, and Science Direct. Key terms used in the search included: "hospice nurse," "palliative," "burnout," "code lavender," "compassion fatigue," "exhaustion," "patient satisfaction," "employee satisfaction," "crisis intervention," "meditation," "debriefing tools," "traumatic stress," "resilience," and "nurse turnover." The search was limited to peer-reviewed articles published between 2000 and 2022. Some articles published before 2017 were included to provide historical background on stress, burnout, and debriefing tools. The Project Lead retrieved 883 research articles relevant to the project. The Project Lead carefully examined these articles for duplicates and relevance to the topic of this project, which resulted in excluding 420 articles. The remaining 463 articles were included in the review (Appendix B; Appendix C; and Appendix D).

This review of the literature (ROL) presented a synthesis of current literature and burnout among its impact on nurses, patients, and healthcare institutions. The synthesis also included evidence-based interventions to assist nurses who had experienced work-related traumatic events. The following terms were used in the DNP project and are defined as follows: Burnout is described as physical and emotional exhaustion leading to negative feelings towards patients and work, callousness to patients, and compassion fatigue (Keidel, 2002; Kelly et al., 2019; Mudallal et al., 2017). Compassion fatigue is a higher level of burnout that leaves hospice staff overwhelmed, causing mental and physical distress (Al-Majid et al., 2018; Kelly et al., 2019). Secondary traumatic stress, interchangeably used with traumatic stress, results from burnout and

compassion fatigue. Secondary traumatic stress is the inability to provide compassionate care because of continuous exposure to stressful events at work (Al-Majid et al., 2018; Kelly et al., 2019). Exhaustion occurs when an individual's mental and physical state is negatively impacted due to burnout, compassion fatigue, and repeated exposure to traumatic stress (Al-Majid et al., 2018; Cross, 2018; Keidel, 2002). Characteristics of Exhaustion consist of an individual who is adverse in their work and is calloused in their feelings and concerns towards their patients, which in turn affects patient safety and quality of care (Cross, 2018; Keidel, 2002; Mudallal et al., 2017).

There are several ways to address a traumatic event after experiencing one. Respite rooms, self-care, yoga, mindful meditation, and forming a debriefing meeting are examples of interventions today in acute care settings (APA, 2022; Cocchiara et al., 2019; Gregory, 2021; Hu et al., 2022). Hospice agencies that provide community services are challenged to designate areas for respite. Providing immediate support can be challenging since the staff functions independently in the community. Thus, forming a debriefing team and having the goal of supporting the staff in mind demonstrated the organization's attempt to provide support during their traumatic event experience.

Secondary Traumatic Stress and Burnout

Secondary traumatic stress is an emotional response to the repeated work-related experience of abnormal or distressing events (APA, 2022; Kellogg, 2020; Tan et al., 2018; Youn & Halfond, 2019). Traumatic stress causes a disequilibrium, affecting hospice nurses' physiological and psychological well-being (Xue et al., 2021). Hospice nurses in community settings isolate themselves from their colleagues and limit their access to supportive resources. Limited access to support resources can lead to the inability of hospice nurses to process the

physiological and psychological aspects of their job. Instead, the hospice nurses are left going from one patient visit to the next without coping with the previous traumatic experience of a patient's death. This lack of access to support services places the hospice nurse at an increased risk for burnout and compassion fatigue. During this time, hospice nurses may learn how to cope with traumatic stress by emotionally distancing themselves from it. This method of coping has been described as a valued professional skill; however, research has shown that it is ineffective and can lead to the hospice nurse leaving the organization due to burnout (Ingebresten & Sagbakken, 2016).

Occupational stressors in nursing include long work hours, poor leadership style, patient loss, emotional distress, caring for unstable patients, supporting family members, or managing the family's pain and grief (Botha et al., 2015; Kellogg, 2020). These occupational stressors are magnified for Hospice nurses leading to traumatic stress (Kelly et al., 2019). Traumatic stress may present as compassion fatigue, negatively impacting job satisfaction and patient health outcomes. Compassion fatigue is an individual's state when experiencing repeated exposures to terminal patients, loss, grief, and shared traumatic events (Al-Majid et al., 2018; Matey, 2016).

Multiple studies have suggested increased nurses experiencing numerous traumatic events, leading to burnout (Al-Majid et al., 2018; APA, 2022; Curry, 2021; Davidson et al., 2017; Kellogg, 2020; Youn & Halfond, 2019). Burnout is a physical and emotional symptom resulting from severe stress, exhaustion, and inability to cope with work-related traumatic events (Mudallal et al., 2017; World Health Organization, 2019). In many articles, Traumatic stress and burnout are used interchangeably to describe energy depletion, exhaustion, decreased compassion, decreased ability to provide high-quality care, increased sick calls, inability to sleep, anger, and hopelessness (APA, 2022; Kellogg, 2020; Youn & Halfond, 2019). These signs of

traumatic stress and burnout can lead to nurses quitting the profession altogether. Physical burnout signs include being tired or drained, suffering from frequent illnesses, frequent headaches or muscle pain, and changes in appetite or sleep habits (Mayo Clinic, 2022). Emotional signs of burnout include a feeling of failure, self-doubt, helplessness, and being trapped or defeated (Mayo Clinic, 2022). burnout's physical and emotional toll can impact a Nurse's life.

Traumatic Events: Impact on Nurses

The impact of secondary traumatic events on nurses can produce emotional and physical reactions such as fear, oversleeping, and avoidance (Harker et al., 2016). Negative physiological and psychological factors associated with traumatic events include stress, burnout, fatigue, exhaustion, and repeated trauma from witnessing death and dying (Aburn et al., 2016; Cooper et al., 2021; Hu et al., 2022; Ledesma, 2015). While in the hospital, the nurses are in a controlled environment where they can receive immediate assistance after experiencing a Code Blue, Rapid Response, or any unusual events. Compared to the acute care setting for hospital nurses, hospice nurses work in the community.

As a result of not having a support system readily available, many hospice nurses can be calloused to the numerous patients they witness suffering, death, and dying (Keidel, 2002; Kellog, 2020; Ingebretsen & Sagbakken, 2016). Increasing nurse resiliency is necessary to decrease the adverse effects of constant exposure to seeing suffering, death, and dying (Aburn et al., 2016; Cooper et al., 2021; Hu et al., 2022; Ledesma, 2015). To successfully care for hospice nurses, leaders need to take time and coordinate the nurses' schedules and patient visits to find a standard meeting time.

Traumatic Events: Impact on Patients

A meta-analysis revealed that burnout and patient safety was more than 60% associated with one another (Garcia et al., 2019). Burnout can lead to poor judgment, inadequate assessments, medical errors, delay in care, and discomfort during patient death (Garcia et al., 2019; Hu et al., 2022; Ingebreetsen & Sagbakken, 2016). A high level of burnout will lead to poor patient outcomes, decreased patient satisfaction, and increased patient complaints (Garcia et al., 2019; Hu et al., 2022).

Traumatic Events: Impact on Organizations

Organizational support is needed to help hospice nurses cope with the traumatic events they experience to prevent them from exiting the organization (Cooper et al., 2021; Kelly et al., 2021). According to the 2021 Nursing Solutions Incorporated National Health Care Retention and RN Staffing Report, training one registered nurse costs \$40,038 (Colosi, 2022). This cost can quadruple if multiplied by the number of nurses leaving one organization. The impact of losing multiple nurses in one organization can lead to poor quality of patient care and higher operational costs for the organization (Becker's Hospital CFO Report, 2021; Colosi, 2022). Addressing burnout is necessary to mitigate staff turnover rates, poor safety practices, and negative financial implications.

Strategies to Assist After Traumatic Events

An abundance of literature supports using debriefing tools to support nurses experiencing work-related traumatic events (Davidson et al., 2017; Malcolm & Tallian, 2018; Stone, 2018). However, there needs to be more information on the application of such tools in community-based hospice and palliative care settings. Debriefing methods like Code Lavender and the seven Phases of Debriefing Tool have successfully addressed work-related traumatic events in acute

care settings (American Counseling Association's Traumatology Interest Network [ACATIN]; Stone, 2018). Thus, the Debriefing Protocol used for this DNP project supported hospice nurses experiencing a traumatic event by providing respite to decrease burnout and compassion fatigue (ACATIN, 2011; Davidson et al., 2017; Gregory, 2021; Stone, 2018).

This Hospice Agency is constantly challenged with the hours nurses are expected to complete their job and tasks while balancing patient and family experience and satisfaction. Reducing secondary traumatic stress can be accomplished by improving work hours and providing relaxation, time, and support to hospice nurses (Hotchkiss, 2018). Moreover, for this Hospice Agency to minimize patient impact, immediate help should be made available for the leaders to identify immediate debriefing needs for their staff.

The Project Lead created an Algorithm to assist and guide the Clinical Manager or AOC and the Nurse experiencing the trauma when the nurse triggers a phone call and reports the event. A similar process called Code Lavender has been successfully used in the acute care setting. Code Lavender is a holistic crisis intervention approach used to respond to and help any staff member in crisis (Davidson et al., 2017; Stone, 2018; Gregory, 2021). Multiple variations of the Code Lavender are being used nationwide to respond to traumatic event crises among nurses. Since Code Lavender has been used successfully in the acute care setting, as evidenced by the multiple adaptations in different organizations, its model guided the Project Lead in creating an algorithm applicable in a community-based setting. These strategies used in an inpatient or acute care setting have inspired the development of the debriefing protocol developed for this DNP project. All discussions about the debriefing protocol will be under the Description of Innovation section.

Description of Innovation

The Debriefing Protocol

The algorithm created by the Project Lead was reviewed and finalized in collaboration with the Ethics Taskforce. The Ethics Taskforce comprised a clinical director, a clinical nurse manager, a frontline nurse, the Ethics Director, a chaplain, and a LCSW. The algorithm is a decision tree that consists of questions designed to help the Clinical Managers, AOC, and nurse experiencing the trauma to assess the demeanor and stress response of the nurse experiencing a work-related traumatic event. Answers to the prompts and the two decision points in the algorithm will help the Clinical Manager or the AOC determine whether activating the Debrief is needed. The steps of the algorithm in Appendix E will show the approved version of the algorithm for the Managers and AOC.

The Debriefing Tool is a seven-phase structured intervention that provides a safe space for individuals to discuss the emotions and distress they experience after a traumatic event (ACATIN, 2011). In 1974, Dr. Jeffrey Mitchell created the debriefing tool used in the current project to respond to a larger project to minimize the emotional impact on emergency workers (Everly & Mitchell, 1995). The Project Lead and Ethics Taskforce adapted four phases of the debriefing tool from the Critical Incident Stress Debriefing (CISD).

The Algorithm and Decision Points

The first step in the debriefing protocol is determining the needs of the nurse experiencing the traumatic event using the algorithm. The Clinical Manager or the AOC and the Nurse experiencing a traumatic event will use the algorithm to determine if a complete debrief is necessary or what level of support needs to be provided to the nurse. Other support levels could include 1:1 conversation with the nurse leader, confidential counseling, speaking with a

bereavement coordinator, or taking time off work. The Clinical Manager or AOC and Nurse will use the algorithm prompts for its first decision point. Based on the answers, this decision point will support a "yes," and the following steps to activate a Debrief will be initiated.

Following the steps for the decision point under "yes," the Clinical Manager or AOC will determine the debrief participants and Ad Hoc participants. The participants are identified hospice staff that took part in a patient's care or were identified as experiencing the traumatic event. The Ad Hoc participants are further described in the Hospice and Palliative Care Team Guidance Tool section. After the participants are identified, the next step will be to set a debrief 24- 48 hours after the initial report of the traumatic event. This time frame allows the nurse to overcome the shock related to the traumatic event. A Microsoft Teams link for the meeting will be sent to the direct care team by the Clinical Manager or AOC via a calendar invite.

The second decision point is where the nurse denies the need for activating the Debriefing Tool. If the answer is determined to be a "no," formal debriefing is unnecessary, and the nurse will receive additional support and resources. However, if the nurse needs extra help, the clinical manager or AOC can return to activating the debriefing process. A follow-up with the Hospice Agency nurse will occur 30-60-90 days after the traumatic event. The timeframe for the follow-up will reinforce to staff that the leadership team is invested in their well-being and recovery from their traumatic event.

The Hospice and Palliative Care Team Debrief Guidance Tool

The Hospice and Palliative Care Team Debrief Guidance Tool, also called the Debrief Guidance Tool, is used at the manager's or AOC's first decision point. The "goal" of debriefing is defined. The following line provides the Manager or AOC considerations for a formal debrief. If a formal debrief is identified, guidance to invite individuals who were part of the care team and

ad hoc individuals like the medical director, regional ethics director, executive leaders, or an employee assistance program representative would fall in this category. The manager or AOC makes the arrangements for a debrief. Under the guidance tool, when the manager of AOC makes the phone call to the participants, the debrief objectives are described. Lastly, if an informal debrief is warranted rather than a formal debrief, resources for additional caregiver support are listed to provide the nurse experiencing the traumatic event. Appendix F shows the Hospice and Palliative Care Team Debrief Guidance Tool.

The Debriefing Tool

The original seven phases of the CISD included: Introduction, Facts, Thoughts, Reactions, Symptoms, Teaching, and Re-entry (ACATIN, 2011). The Project Lead and the Ethics Taskforce decided that four phases of the seven-phase Debriefing Tool were beneficial to include in the debriefing protocol. The first three phases of the Debriefing Tool were bundled as the first phase. The intention of bundling the phases was to be time efficient because the individuals in the debrief belong to one institution where staff is familiar with one another. The four-phase debriefing tool aims to validate the psychological and emotional stress nurses experience from the traumatic event, hoping to decrease burnout (ACATIN, 2011; APA, 2022).

The second page of the Debrief Guidance Tool is the Hospice and Palliative Care Debrief Process. In the Active Phase of the Debrief, Ground Rules are shared at the open. The facilitator uses the debriefing tool to guide the nurses in sharing and exploring their experiences with the rest of the care team (CHHA, LCSW, Chaplain, and Providers). The first phase is the roundtable. Each member will reflect on their roles, thoughts, and feelings about the traumatic event. The second phase is the reaction and symptom; this phase aims to understand the staff's response after the event and for the leaders to identify signs of burnout and trauma. The third phase is

teaching and re-entry. The leaders need to determine if there were any takeaways or identified gaps that the team could do differently or similar situations and how that event was treated.

Lastly, the fourth phase is closing. This portion is to reflect and make any last statements that may have yet to be addressed during the initial phases of the debriefing process. The debriefing timeframe can be scheduled anytime between 24 to 48 hours after the traumatic event. The debriefing can last up to three hours and is optimal within a group of twenty or fewer (Appendix G).

Methods

The purpose of this DNP project was to examine if a Debriefing Protocol activated following a job-related traumatic event would decrease overall burnout, exhaustion, and disengagement in hospice nurses. This DNP project involves creating and implementing an algorithm to help Clinical Managers and AOCs when the affected nurse reaches out after a traumatic event and creating a visual guide for nurses to understand when to activate the debriefing protocol. The manager or AOC then goes through the Debrief Tool questions to identify the level of support the nurse needs. The purpose was to support nurses experiencing burnout due to job-related traumatic events.

Design

A pre-test and post-test design was used to measure exhaustion among participants using the Oldenburg Burnout Inventory (OLBI) (Demerouti et al., 2001). A detailed description of the OLBI survey is provided under the instrument section. Appendix H is a copy of the OLBI. Used for the pre-and post-test intervention before and after implementing the debrief protocol.

Setting

This project was conducted at a not-for-profit Hospice Agency in Torrance, California. The Hospice Agency is a community-based agency that provides end-of-life care and serves patients with six months or less to live. The project agency assigns patients to care teams once admitted to Los Angeles County and Orange County. Hospice services are provided to patients in private homes, skilled nursing facilities, assisted living facilities, boards and care homes, and hospitals. The number of patients seen yearly is 2,000 adults and 300 pediatric patients, with an average daily census of 290.

The Hospice Agency provides 24-hour services, which are provided in three shifts: day, evening, and night. The nurse director and Clinical Managers are on call 24 hours daily. The agency staff consists of six medical physicians, three Nurse Practitioners, one Nurse Director, four Clinical Managers, 25 Registered Nurse Case Managers (RNCM), 30 Field Registered Nurses (FRN), 20 Licensed Vocational Nurses (LVNs), 20 Certified Home Health Aides (CHHA), 18 LCSW, six liaisons, six chaplains and an evening assigned AOC.

Sample

The participants in this project included all nurses working in the Hospice Agency. As for the debrief protocol, the entire multidisciplinary direct care team, which consists of all providers assigned to the patient, will be included in the debriefing. The multidisciplinary direct care team includes a physician, nurse practitioner, Clinical Manager, RNCM, FRN, LVN, CHHA, LCSW, and a chaplain. Each care team member plays a role in caring for and supporting patients through their end-of-life journey. The RNCM is the lead for the care team. The FRN is the frontline nurse assigned to the patient and works daily with patients and their families. The LVN follows patients who need pain management, continuous care, or wound care. The LCSW and Chaplain provide support resources on preparing the patient for end-of-life, bereavement, coping, and healing touch for the patient and family. For this project, data was collected from Hospice nurses only.

The nurses were informed that participation was entirely voluntary and there would be no ramifications for lack of participation. Information, updates, and reminders about the Debriefing Protocol were provided via emails and staff meetings. The formal education occurred in the nurses' specific and clinical leadership team meetings using Microsoft Teams. All potential participants received instructions regarding the project's purpose, the targeted participants, and

the duration of the piloted project. A frequently asked questions (FAQ) sheet was provided to the nurses to help them absorb the provided education. The nurses were informed that participation was voluntary and that clicking on the RedCAP link provided consent to participate in the DNP project. The Frequently Asked Questions (FAQ) sheet clearly states no repercussions for anyone who will not participate in the Debriefing Protocol project.

Staff Education

Before implementing the Debriefing Protocol, the Project Lead conducted multiple in-service sessions to educate the Clinical Managers, AOCs, nurses, and direct care teams on the purpose and use of the Debriefing Protocol. The education included sharing the algorithm guidance for the managers and AOCs and how the visual was different for the nurses. The education had a set time limit of 30 minutes, with 20 minutes of information and 10 minutes for questions and answers. A 30-60-90-day follow-up with the affected individual or individuals will occur with the manager.

Procedures

Approval Process and Ethical Considerations:

The University of Iowa Hospitals and Clinics granted the Project Lead permission to use the Iowa Model for this DNP project (Appendix I). The permission to use the seven phases of debriefing is also shown in Appendix J. The institutional approval letter to use the project site is available in Appendix K. Following approval from the Ethics Taskforce, the Debriefing Protocol was submitted for review. It was approved by various committees of the Hospice Agency, including the local and regional Ethics Committees, the Clinical Leadership Team, and the Senior Executive Team. The Institutional Review Boards (IRB) at CSUF, and the piloting Hospice Agency approved this project. Participants consented to participate by reading the

project's full description and taking the surveys. Responses were deidentified to maintain the confidentiality of the nurses participating in the project. The OLBI results were stored and tracked in RedCAP, the tool for gathering data by the Hospice Agency. The pre-and post-data from the participants was kept on RedCAP for comparative data.

The saved data was stored on the Project Lead's password-protected laptop and placed in a locked cabinet in a locked office. The Health Insurance Portability and Accountability Act (HIPAA) was followed when debriefing occurred on a traumatic patient event. HIPAA requires that all patient information is kept confidential and protected.

Survey Timeframe

The Project Lead made the OLBI available to the participants on Friday, October 28, 2022, through Friday, November 11, 2022, to gather baseline data on burnout. A reminder was sent at 0700 daily to remind participants to complete the OLBI survey until the closing November date. The pre-survey results provided baseline data for the Hospice Agency participants' burnout level. After three months of piloting the Debrief Protocol, the participants received a post-data OLBI email. Data collection for the post-survey began on Saturday, January 28, 2023, and continued through Saturday, February 11, 2023. A reminder was again sent at 0700 daily to remind the participants to complete the post-OLBI until the February closing date.

Measures

The Project Lead chose the OLBI to measure overall burnout, exhaustion, and disengagement experienced by nurses before and after using the Debriefing Protocol. Evangelia Demerouti and Friedhelm Nachreiner introduced the OLBI in Europe in 1998 (Demerouti et al., 2001; Sinval et al., 2019). The OLBI is an accepted alternative measurement for burnout and

engagement; instead of the Maslach Burnout Inventory because of its reliability, validity, and internal consistency.

The OLBI is a valid and reliable instrument for measuring burnout with Cronbach's alpha of 0.81 (Reis et al., 2021; Sinval et al., 2019; Tipa et al., 2019). Various occupational studies in the literature have shown internal consistency with results in burnout and the level of measurement in disengagement and the level of measurement in exhaustion using the OLBI (Demerouti et al., 2001; Sinval et al., 2019). The OLBI consists of 16 statements rated using a 4-point Likert scale, with 1 being strongly agrees and 4 being strongly disagree. The OLBI measures disengagement with questions: 1, 3, 6, 7, 9, 11, 13, and 15, while exhaustion is measured by questions: 2, 4, 5, 8, 10, 12, 14, and 16. The OLBI total score ranges between 35 through 64. The higher the scores in Disengagement and Exhaustion, the higher the burnout (Demerouti et al., 2001). Disengagement and exhaustion are believed to be the core causes of burnout (Demerouti et al., 2001; Sinval et al., 2019; Tipa et al., 2019).

The specific aim was: Using the debriefing protocol would decrease a) overall burnout as measured by the OLBI, b) decrease exhaustion and c) decrease disengagement. The post-OLBI gathered in RedCAP also measured staff perception in the burnout level based on the pre-data collected from October through November 2022 and post-data collected at the end of January 2023. The OLBI survey provided insight into the effect of the Debriefing Protocol on the hospice staff within the first three months. It offered the Project Lead insight into the created intervention that provides the support staff needs after experiencing a traumatic event.

The success and efficacy of the Debriefing Protocol will be evaluated by comparing the Willis Towers Watson Survey results from 2022 and 2023. The results for burnout and intent to stay with the Hospice Agency organization from the Willis Towers Watson Survey will not be

resulted until January 2024. The effectiveness of the Debriefing Protocol will be assessed by comparing the pre-and post-implementation of the Debrief Protocol on burnout and intention to leave.

Timeline

The Project Lead reviewed the literature from March through April 2022 to identify debriefing tools, stress, burnout, compassion fatigue, exhaustion, and trauma. The literature review also provided information on evidence-based practices that decreased the above concerns. From May through June 2022, the Project Lead confirmed the Project Site and obtained approval of support from the Project Site Sponsor. From July through September 2022, the Project Lead prepared a presentation, defended this DNP Project, and started the process for approval from CSUF and Hospice Agency IRB. From October through November 2022, the Project Lead taught managers, AOCs, and Hospice Agency staff. From November through January 2023, the project's pre- and post-data were collected to analyze and finalize. Lastly, in April of 2023, the Project Lead will disseminate and defend this project. Appendix L shows the DNP project timeline.

Results

Data Analysis

All analyses were completed using the IBM Statistical Package for Social Sciences (SPSS) Version 28.0.0.0. A total of 41 participants, including 13 LVNs, 11 ADNs, 14 BSNs, and 3 MSNs, completed the OLBI. We compared overall burnout, exhaustion, and disengagement before and after using the Debriefing Protocol intervention. All comparisons were performed using dependent sample *t*-tests with Bonferroni corrections with a significance level set at $P < 0.05$.

Results

A total of 41 participants, including 13 LVNs, 11 ADNs, 14 BSNs, and 3 MSNs, participated in this project. They completed the pre-test, the Debriefing Protocol, and the post-test. Appendix M is for the demographic characteristics of the participants.

There was no difference in the incidence of burnout between the pre-test following the Debriefing Protocol ($M = 36.37 \pm SD = 6.84$) was not different than the pre-test ($M = 36.34 \pm SD = 7.07$), $t(40) = -.025$, $p > 0.05$) and post-test ($M = 36.34 \pm SD = 7.07$), $t(40) = -.025$, $p > 0.05$), Appendix N. Exhaustion decreased following the Debriefing Protocol ($M = 18.07$, $\pm SD = 3.84$) compared to before the Debriefing Protocol ($M = 19.27$, $\pm SD = 3.702$), this difference approached statistical significance ($p = 0.087$), Appendix O. There was no statistically significant difference in disengagement between the pre-test ($M = 17.07$, $\pm SD = 4.15$) and post-test ($M = 18.29$, $\pm SD = 3.72$), Appendix P for results.

Discussion

Evaluation

The Debriefing Protocol used in this project was designed based on strategies discussed in the literature. It was expected to decrease burnout, exhaustion, and disengagement among nurses in the project's Hospice Agency. The intervention results approached significance in both exhaustion ($P= 0.08$) and disengagement ($P= 0.07$) but did not reach significance. While such a strategy improved outcomes related to burnout in other studies (Knott et al., 2023; Lobb et al., 2010), our burnout results showed no significant effects of the Debriefing Protocol. Many variables could have contributed to our null findings. In the months leading to and during the implementation of the Debriefing Protocol, internal and external factors may have contributed to the results.

The Hospice Agency started to admit more patients into their service that were acute or chronically intubated. The hospice nurse would follow the patient from their starting point (hospital or facility) and drive to the patient's destination, which often was the residence. The hospice nurse would receive the patient from the ambulance and then assess the comfort level and stability upon arrival. While waiting for the hospice physician to arrive, the hospice nurse prepares the family, re-educating them on the process and expectations once the extubation begins. Even if there were any particular rituals, prayers, or cultural practices before extubation, the hospice nurse remains present. Although experiencing the loss of a patient is part of the hospice specialty, multiple exposures to compassionate extubation and having to explain the uncertainty of when the patient may pass is challenging for the hospice nurse.

Before implementing the debriefing protocol, the pediatric hospice team had a general form of debriefing as part of their weekly interdisciplinary team meetings. Studies completed in

other organizations have shown that debriefing sessions are needed to promote emotional support, compassion, and a connection to their patients (Davidson et al., 2017). The focus was on experiences and perspectives rather than the direct impact of specific patients or traumatic events. Since a general debrief was part of the weekly pediatric team meeting, it is possible that relearning to debrief on a particular traumatic event may have confused the intended delivery of the debriefing protocol.

Another factor that may have affected the hospice nurses was the rapid growth of the pediatric hospice program. The program's progression caused a lot of pressure on the nursing team in the project's facility. The census nearly doubled for the team, and the hospice nurses struggled to provide the family with the same high-quality care and meaningful time. To assist with staffing shortages, adult hospice nurses who volunteered to assist the pediatric team were placed in education and training content squeezed into two weeks when the specialty training is up to a year. The pediatric hospice nurses felt they could not ease the family's way while experiencing the loss of a child. Due to the increased census and limited staff, the hospice nurses experienced more pediatric deaths and were stretched with resources. As a result, several nurses left the organization due to increased burnout, leaving the remainder of the team more pressured and defeated.

This project occurred at the beginning of the winter season. During this time, the patient census peaks. Additionally, while implementing the Debriefing Protocol, participants experienced several holidays, Halloween, Thanksgiving, Christmas, and New Year's. The winter season is the busiest, and it is possible that participants were not focused and open to the education and implementation of the project. Along with celebrating these holidays, other participants were going through personal hardships. Some nurses had parents placed in hospice

while caring for their own terminally ill patients; some nurses had children suffering from mental health problems, and some were going through the death anniversaries of loved ones.

More than 50% of the participants in this DNP project had ten years or less of hospice experience, possibly contributing to the higher disengagement and exhaustion leading to burnout. This finding was similar in Australia, where nurses with ten years or less experience in the community setting were more susceptible to burnout (Lobb et al., 2010). This Australian study assessed informal and formal debriefing, which showed some effect but was insignificant (Davidson et al., 2017; Lobb et al., 2010). Although the Debriefing Protocol used in this project did have both informal and formal debriefing available, no informal debriefings were reported to the Project Leader.

Limited studies and research are available to address traumatic events occurring in community-based settings like hospice care. With independent hospice staff and limited access to direct support after a traumatic event, leadership must find a solution to mitigate these stressors. Other considerations affecting results are the increasingly complex understanding of patients' best practices to address traumatic events in acute care settings where immediate resources and designated respite rooms are available for staff experiencing traumatic events.

Limitations

This DNP project's limitation was identified as the Fidelity to Intervention Problem. The timeframe of the provided education to implement the intervention was limited to several weeks, which did not give enough time for the participants and the project facilitator to engage in the purpose and goal of the Debrief Protocol. The project composition sample of demographics was with less than or equal to five years of hospice experience, which can lead to increased burnout. At the time of implementation, there was an increase in patient census resulting in staffing

shortages, leading to staff turnover. This project was focused on only one institution. Improving the debriefing protocol will allow other institutions to roll out this process.

Recommendations

The outcomes of this DNP project provided insight into the organizational support leaders need to provide to support hospice nurses experiencing traumatic events. It has been demonstrated that addressing traumatic events for nurses will decrease stress and burnout, increase satisfaction, increase job retention, increase patient satisfaction, and increase social support. Evidence has shown that implementing interventions in the workplace improves nurses' mood, increases their empathy, and reduces their stress and burnout, which leads to improved quality of life for nurses (Davidson et al., 2017; Lobb et al., 2010).

A nurse's ability to cope with the adversities in the workplace environment can lead to decreased stress and burnout. Re-education on using the algorithm, debrief guidance tool, and debriefing phases to improve implementation of the Debriefing Protocol can improve adherence to the debrief. There is a need to retrain the Managers, AOCs, and Nurses on the debriefing protocol. Incorporating participant feedback and recruiting more debriefing facilitators can increase engagement in a practice change. After, there needs to be an observation to ensure that the intervention is delivered as intended. When the Project Lead feels the staff are well versed with the debriefing protocol, the study must be replicated using the OLBI for the pre-test and post-test.

Conclusion

Interventions such as the debriefing protocol have been shown in the literature to decrease burnout. However, the debriefing protocol in this project showed no overall difference in decreasing burnout. When a crisis or traumatic event occurs, immediate care team members

mask their cognitive and emotional stressors by going through their job expectations (Al-Majid et al., 2018; Curry, 2021). Nurses with no outlet to process their emotions and thoughts increase their traumatic stress and burnout (ACATIN, 2011). The purpose of the Debriefing Protocol was to create an effective algorithm and implement the Debriefing Tool. The Debriefing Protocol provides a structured guideline to allow hospice nurses to process their emotions and thoughts without fear of judgment or blame. As nurses can process traumatic experiences through debriefing, their stress and burnout decrease. Leaders must address the hospice nurses' repeated exposure to traumatic events to optimize recovery and decrease burnout, compassion fatigue, stress, and exhaustion. In return, patients will have a safe and increased quality of care. It is imperative to continue to test strategies to combat burnout among nurses. Successful strategies can provide hospice nurses support and a safe space to become vulnerable and learn from their experiences.

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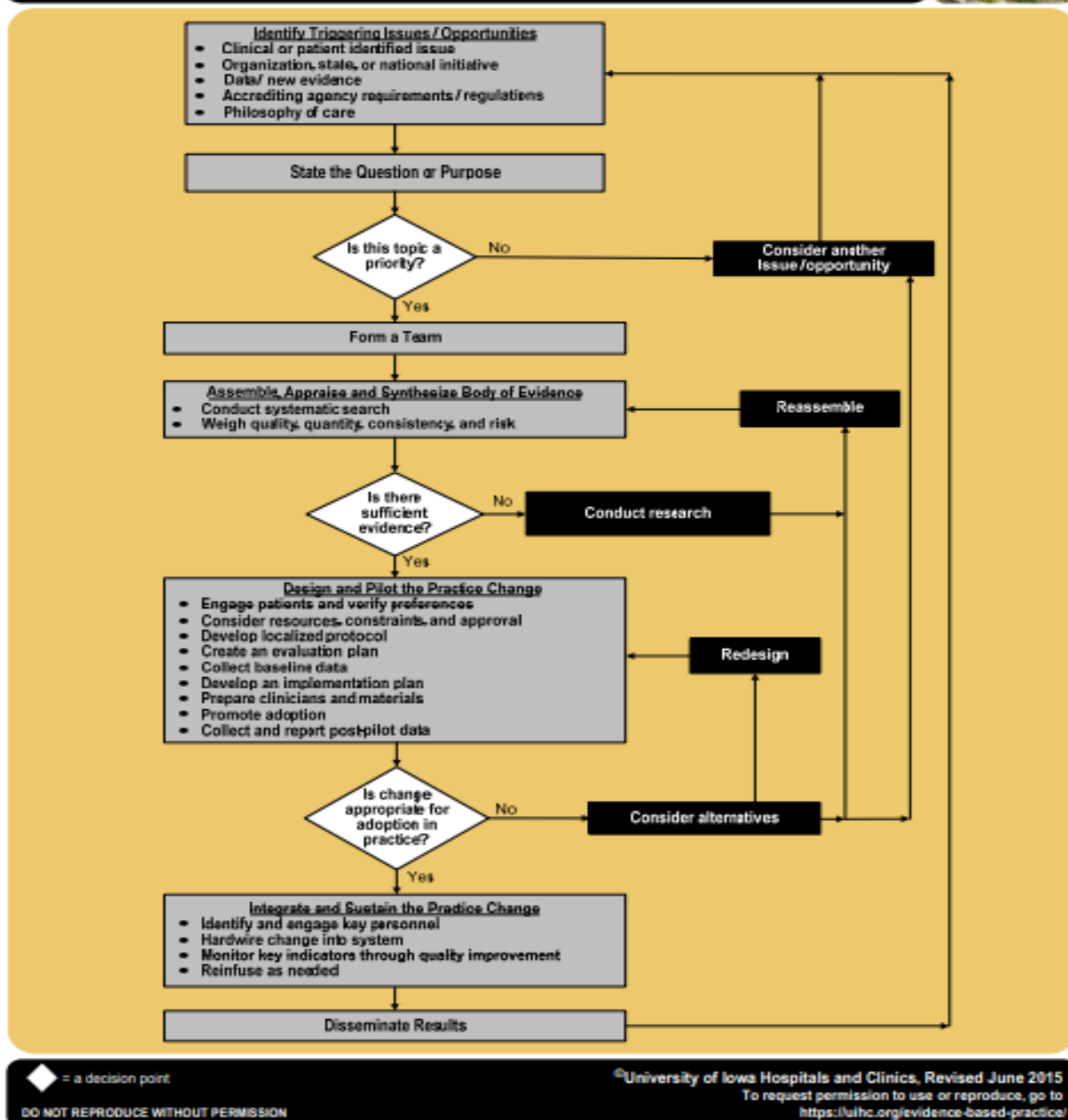
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Appendix A

The Iowa Model

The Iowa Model Revised: Evidence-Based Practice to Promote Excellence in Health Care



Appendix B

Articles Searched from CINAHL/EBSCO

Terms	Limitations	# of Articles Retrieved	# of Articles Excluded	# of Articles Reviewed	Articles Used
Nurse Resilience and Burnout	Peer-reviewed, Years: 2020 to 2022	52	2	10	7
Burnout and Stress	Peer-reviewed, journal article, source type, all adults, Years: 2017 to 2022	41	5	8	6
Crisis intervention, meditation	Peer-reviewed, journal article, source type, Years: 2017 to 2022	23	10	5	4
Hospice nurses and stress and burnout	Peer-reviewed, journal article, source type, Years: 2017 to 2022	61	5	7	3

Appendix C

Articles Searched from Science Direct

Terms	Limitations	# of Articles Retrieved	# of Articles Excluded	# of Articles Reviewed	# of Articles Used
Nurse Resilience and Patient Satisfaction	Peer-reviewed, Nursing and Health Professions Years: 2021 to 2022	67	10	4	2
Burnout and Occupational Stress	Peer-reviewed, Nursing and Health Professions Years: 2017 to 2022	72	5	12	6
Code Lavender and Crisis Intervention	Peer-reviewed, Nursing and Health Professions Years: 2017 to 2022	46	2	10	5
Hospice nurses and traumatic stress	Peer-reviewed, journal article, source type, Years: 2017 to 2022	236	160	25	3

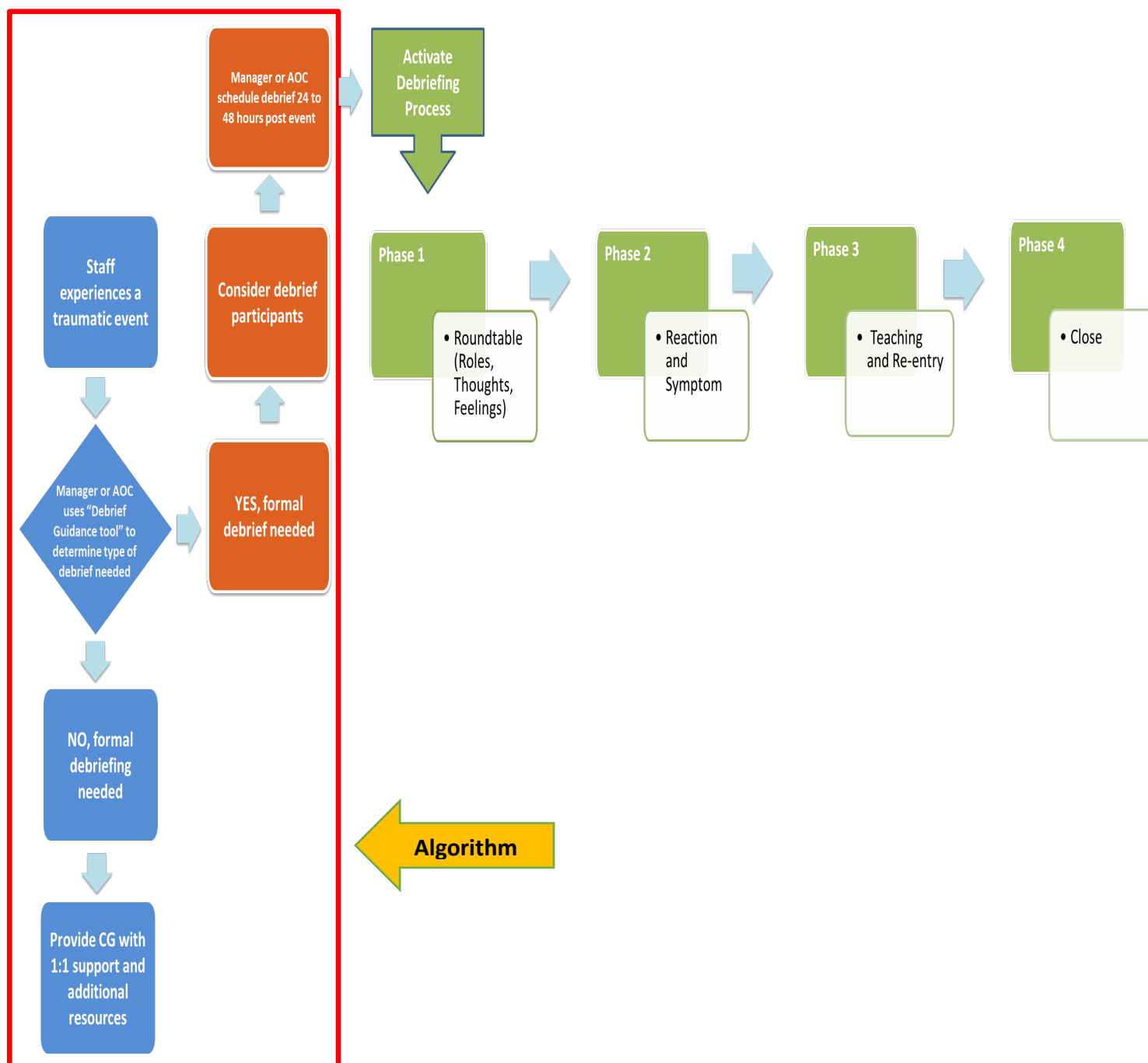
Appendix D

Articles Searched from PubMed

Terms	Limitations	# of Articles Retrieved	# of Articles Excluded	# of Articles Reviewed	# of Articles Used
Hospice Nurse and Patient Satisfaction	Peer-reviewed, Years: 2017 to 2022	53	26	10	5
Burnout and debriefing tool	Peer-reviewed, full text, RCT, Systematic Review, Years: 2017 to 2022	148	139	9	4
Meditation and Employee Satisfaction	Peer-reviewed, Years: 2017 to 2022	42	36	6	3
Hospice nurses and stress and burnout	Peer-reviewed, journal article, source type, Years: 2017 to 2022	42	20	4	2

Appendix E

The Debrief Protocol: Algorithm for Managers and AOCs



Appendix F

Hospice and Palliative Care Team Debrief Guidance Tool

Hospice and Palliative Care Team Debrief Guidance Tool

Goal: To create a "culture of debriefing" in which debriefing the dynamics and circumstances of providing the care become a regular and reliable process within our team dynamics in support of our caregiver's well-being and support. A debriefing conversation is an opportunity for sharing and processing related to difficult patient deaths or cases.

A formal debrief may be appropriate to consider when:

1. requested by a caregiver or team
2. there is anticipated, witnessed, or reported distress in a caregiving team
 - a) strong emotion when discussing a case
 - b) trauma event
 - c) voices ethical or moral conflict
 - d) high acuity (i.e. palliative sedation, compassionate withdrawal).
 - e) difficult psych-social factors surrounding the case

Driven by the assessment of the need, the following participants should be considered:

1. individual or team who directly managed or cared for the patient
2. ancillary staff that was involved in the case (i.e. Team Administrative Assistant, Liaison, etc.).
3. ministry-designated debrief champions (MSW & Chaplain peer from each ministry)
4. Team Manager
5. patient's hospice or palliative care provider
6. Director or facilitator

Ad hoc participants as the case scenario warrants an assessment:

1. Medical Director, Regional Executive Medical Director, or Chief of Medicine for Hospice
2. Regional Ethics Director
3. Executive Director of Spiritual Health
4. Executive Director of Hospice Care
5. Mission Integration
6. EAP rep

Objectives to holding a debriefing:

1. To provide a safe place to process the emotions of the event.
2. Normalize the experience of processing- "no one cares alone, we are in this together".
3. Active listening and therapeutic presence
4. Educate what a caregiver *may* experience soon after the event and later as processing occurs- "what might I expect and experience"?
5. Connect caregivers to additional resources

Resources for additional caregiver support:

1. Encourage mental-health day off and/or defer assignment of new patient to caseload during caregiver recovery if possible
2. Connect to additional Providence services
 - a) Lyra
 - b) ChooseWell TeleSpiritual Health: <https://www.mychoosewell.org/resources/spiritual-health/>
 - c) Mental Wellness: [Mental Wellness Resources | Choose Well \(mychoosewell.org\)](#)

Appendix G

Hospice and Palliative Care Debrief Process

Hospice and Palliative Care Team Debrief Guidance Tool

Hospice and Palliative Care Debrief Process

Ground Rules to Share at the Open:

- It is not psychotherapy or counseling
- Provides the caregiver(s) the chance to review reactions, emotions, and feelings to an event.
- Confidentiality (What is said here stays here).
- You do not need to speak, if you choose not to, you can pass.
- It may bring up thoughts about a similar incident, we ask that you just speak to this event.

Process:

1. Roundtable the "Role, Thoughts & Feelings" phase

- a) What was your role? How were you connected to the event? What did you see when you encountered the event?
- b) What was your first thoughts and feelings after you heard about the incident? What were your thoughts and how were you feeling after you disengaged from the incident?

2. "Reaction & Symptom" phase

- a) What was the worst part of this for you?
- b) Have you experienced any ~~symptoms~~ since the event? If so, what?

3. "Teaching" & "Re-entry" phase

- a) Normal reactions to an abnormal event, self-care emergence, know your limits- coordinate time off to recover as needed.
- b) If any good can come out of this, what could it be? Lessons learned? Redeeming Value?

4. Close:

- a) invite summary statements and/or reflection
- b) orient to additional resources
- c) thank participants for their transparency, time, and participation

Notes:

- Debrief the "Debriefers"

Appendix H

The Oldenburg Inventory Tool (OLBI)

oldenburg burnout inventory

name:

date:

Instructions: Below you find a series of statements with which you may agree or disagree. Using the scale, please indicate the degree of your agreement by selecting the number that corresponds with each statement.

		<i>strongly agree</i>	<i>agree</i>	<i>disagree</i>	<i>strongly disagree</i>
1.	I always find new and interesting aspects in my work (D)	1	2	3	4
2.	There are days when I feel tired before I arrive at work (E.R.)	1	2	3	4
3.	It happens more and more often that I talk about my work in a negative way (D.R)	1	2	3	4
4.	After work, I tend to need more time than in the past in order to relax and feel better (E.R)	1	2	3	4
5.	I can tolerate the pressure of my work very well (E)	1	2	3	4
6.	Lately, I tend to think less at work and do my job almost mechanically (D.R)	1	2	3	4
7.	I find my work to be a positive challenge (D)	1	2	3	4
8.	During my work, I often feel emotionally drained (E.R.)	1	2	3	4
9.	Over time, one can become disconnected from this type of work (D.R)	1	2	3	4
10.	After working, I have enough energy for my leisure activities (E)	1	2	3	4
11.	Sometimes I feel sickened by my work tasks (D.R)	1	2	3	4
12.	After my work, I usually feel worn out and weary (E.R)	1	2	3	4
13.	This is the only type of work that I can imagine myself doing (D)	1	2	3	4
14.	Usually, I can manage the amount of my work well (E)	1	2	3	4
15.	I feel more and more engaged in my work (D)	1	2	3	4
16.	When I work, I usually feel energized (E)	1	2	3	4

Note: Disengagement items are 1, 3(R), 6(R), 7, 9(R), 11(R), 13, 15. Exhaustion items are 2(R), 4(R), 5, 8(R), 10, 12(R), 14, 16. (R) means reversed item when the scores should be such that higher scores indicate more burnout.

**disengagement
sub-total:**

**exhaustion
sub-total:**

**full scale
total:**

Delgadillo et al (2018) reported "Therapists are identified as having low, medium or high OLBI-D scores, based on scores above or below 1 standard deviation of the mean ($M = 2.15$, $SD = 0.52$; ≤ 1.62 = low, 1.63 to 2.67 = medium, ≥ 2.68 = high)."

Appendix I

Permission to use the IOWA Model

Permission to Use The **Iowa Model** Revised: Evidence-Based Practice to Promote Excellence in Health Care  

  Inbox x

 **Kimberly Jordan - University of Iowa Hospitals and Clinics** <survey-bounce@survey.uiowa.edu> Tue, Mar 22, 9:11 PM   

to me ▾

You have permission, as requested today, to review and/or reproduce *The Iowa Model Revised: Evidence-Based Practice to Promote Excellence in Health Care*. Click the link below to open.

[The Iowa Model Revised \(2015\)](#)

Copyright is retained by University of Iowa Hospitals and Clinics. **Permission is not granted for placing on the internet.**

Reference: **Iowa Model** Collaborative. (2017). **Iowa model** of evidence-based practice: Revisions and validation. *Worldviews on Evidence-Based Nursing*, 14(3), 175-182. doi:10.1111/wvn.12223

In written material, please add the following statement:
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Please contact UHCNursingResearchandEBP@uiowa.edu or 319-384-9098 with questions.

 Reply  Forward

Appendix J

Permission to use the 7 Phases Debriefing Tool

Re: FOR YOUR ATTENTION- Permission to use the 7 phase of process debriefing tool

From: Nancy Driver <ndriver@counseling.org>

Sent: Tuesday, April 26, 2022 8:03 AM

To: Santos, Claire <Cmjuan@csu.fullerton.edu>

Subject: [External] RE: FOR YOUR ATTENTION- Permission to use the 7 phase of process debriefing tool

External Email Use Caution and Confirm Sender

Hello Ms. Santos,

Thank you for your permissions request. I am happy to assist you with this matter. Please consider permission to be granted for educational usage.

Best wishes,
Nancy Driver

From: ACA Ethics <Ethics@counseling.org>

Sent: Monday, April 25, 2022 12:08 PM

To: Nancy Driver <ndriver@counseling.org>

Cc: ACA Ethics <Ethics@counseling.org>

Subject: FOR YOUR ATTENTION- Permission to use the 7 phase of process debriefing tool

From: Santos, Claire <Cmjuan@csu.fullerton.edu>

Sent: Sunday, April 24, 2022 8:12 PM

To: Professional Education <ProfessionalEducation@counseling.org>; ACA Ethics <Ethics@counseling.org>

Cc: dec2cj@gmail.com

Subject: Permission to use the 7 phase of process debriefing tool

Caution: This email originated outside of ACA's network. Please do not click any links or open attachments unless you were expecting the email.

Hello,

My name is Claire Marie Santos, and I am a Doctorate in Nursing Practice student at the California State University of Fullerton.

I wanted to ask for permission to use the 7-phase process for the debriefing method for my project.

My project currently has a concept of debriefing with Nurses after a traumatic or crisis event.

I appreciate your consideration.

Appendix K

Institutional Approval Letter

Institutional Approval Letter



08/05/2022

This letter is to show that, I, Shayna Stiles, as the Executive Director of Providence Trinity Care Hospices, California and Texas at Trinity Care Hospice Torrance, CA, give permission to Claire Marie J. Santos, Dr. Sadeeka Al- Majid, and Dr. Shauna Pearce to conduct the project titled **An Evidence- Based Intervention to Address Traumatic Clinical Events Among Hospice Nurses** on the following department's Providence Trinity Care Hospices.

Upon obtaining all necessary clearances from the Nursing Research Council, (and/or other entities in the facility as needed) at Providence TrinityCare Hospices, and after obtaining the necessary IRB determination/approval, the above-named project lead is allowed to:

- (1) collect data
- (2) access necessary documents/data
- (3) conduct necessary interactions with staff/patients relevant to their project
- (4) distribute the Debriefing Protocol
- (5) provide education to staff
- (6) access to organization Caregiver or Patient Satisfaction surveys

The project lead is responsible for ensuring that all activities related to conducting the project are in compliance with the policies that govern practice, HIPPA, and research and research-related regulations at Providence Trinity Care Hospices and its covered entities.

If you have any questions or concerns, please do not hesitate to contact me.

Signature:

Shayna Stiles

Name: Shayna Stiles

Title: Senior Executive Director, Providence Hospices, California and Texas

Contact Information: 5315 Torrance Blvd.,
Torrance, CA 90503

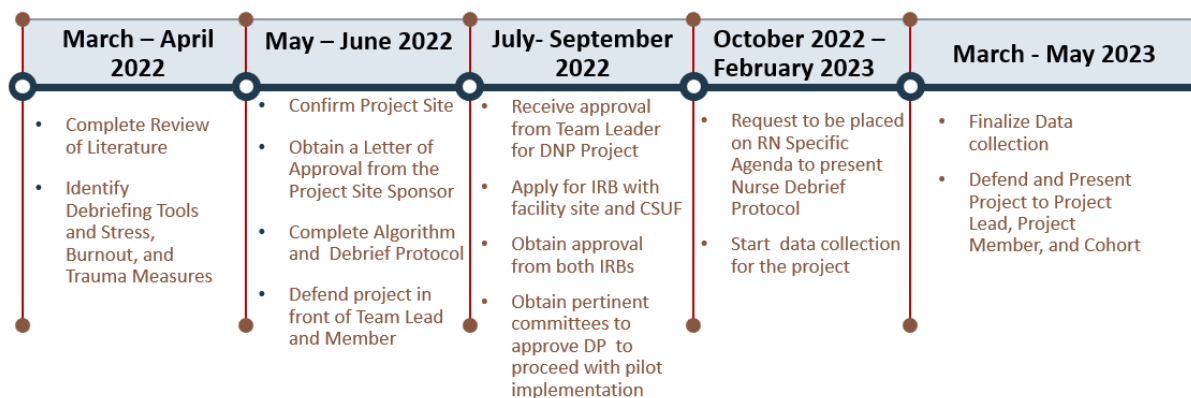
Appendix L

DNP Timeline

DNP Project TimeLine

A NURSE DEBRIEFING PROTOCOL TO ADDRESS

TRAUMATIC CLINICAL EVENTS



Appendix M

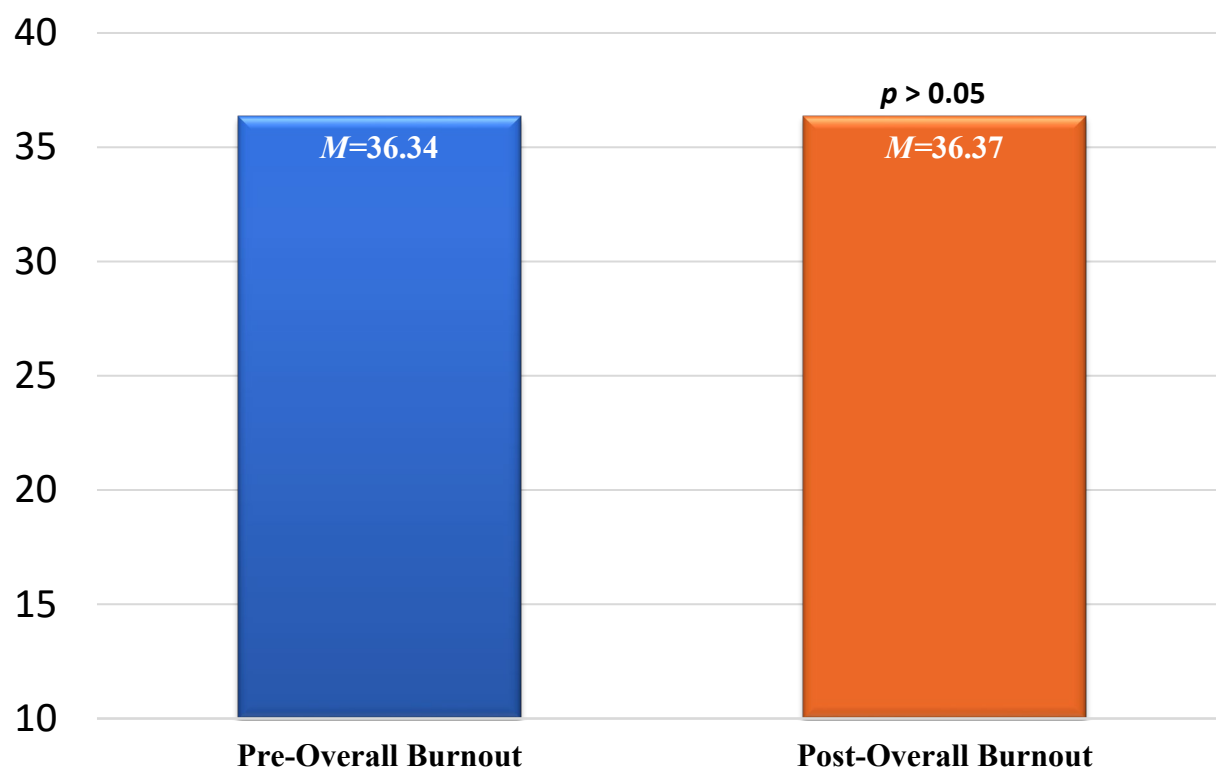
Demographic Characteristics of the Sample

Characteristic	n (%)
Gender	
Female	36 (87.8%)
Male	5 (12.2%)
Age	
≤25	0 (0%)
26 to 35	6 (14.6%)
36 to 45	12 (29.3%)
46 to 55	13 (31.7%)
56 to 65	9 (22%)
66≥	1 (2.4%)
Level of Education	
LVN	13 (31.7%)
ADN	11 (26.8%)
BSN	14 (34.1%)
MSN	3 (7.3%)
Years of Nursing Experience	
≤5	5 (12.2%)
6 to 15	22 (53.7%)
16 to 25	4 (9.8%)
26 to 35	5 (12.2%)
36 to 45	5 (12.2%)

46 ≥	0 (0%)
Years with Organization	
≤5	22 (53.7%)
6 to 15	14 (34.1%)
16 to 25	5 (12.2%)
26 to 35	0 (0%)
36 to 45	0 (0%)
46 ≥	0 (0%)

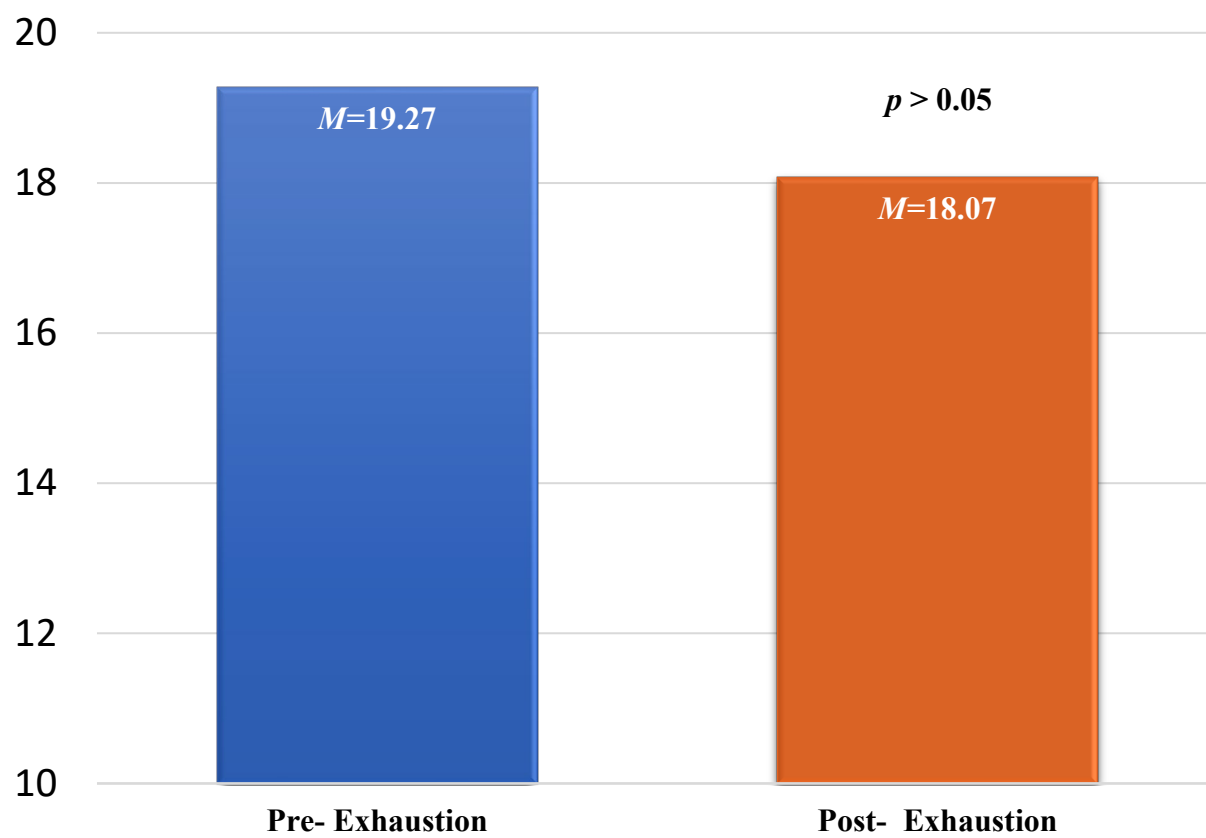
Appendix N

Average Burnout Before and After the Intervention



Appendix O

Average Before and After the Intervention



Appendix P

Average Disengagement Before and After the Intervention

